

Emergency Placement Procedure

SECTION 1: PURPOSE, SCOPE & REFERRAL CRITERIA

1 PURPOSE

This procedure establishes a clear, structured workflow for the emergency admission of children or young persons into Chosanta Management services. It ensures that all emergency placements are managed safely, lawfully, and in the best interests of the child, in compliance with the following legislative and regulatory framework:

- ✓ **Child Care Act 1991, Section 12** — Garda power to remove a child to safety without a warrant where there is an immediate and serious risk to health or welfare
- ✓ **Child Care Act 1991, Section 13** — Emergency Care Order (ECO) granted by the District Court; maximum duration 8 days; application within 3 days of Section 12 removal
- ✓ **Child Care (Placement of Children in Residential Care) Regulations 1995 (S.I. 259/1995)** — Statutory framework governing care plans, supervision, and records for children in residential care
- ✓ **Children First Act 2015** — Mandated reporting obligations; child safeguarding statements; Children First National Guidance 2017
- ✓ **HIQA National Standards for Children's Residential Centres 2018** — Standard 3 (Safe Care and Support); Standard 5 (Leadership, Governance and Management); Standard 6 (Responsive Workforce)
- ✓ **Tusla National Standard Operating Procedure — Special Emergency Arrangements (SEAs)** — Governs provider vetting, staffing, monitoring and transition to regulated placements

Emergency placements arise in circumstances where a child requires immediate accommodation due to a safeguarding concern, family crisis, or breakdown of an existing placement. This procedure is designed to ensure that no child is placed without appropriate risk assessment, safeguarding checks, and staff resourcing — even in time-pressured situations.

2 SCOPE

- ✓ All emergency and out-of-hours referrals received from Tusla, social workers, An Garda Síochána, or other statutory bodies
- ✓ All staff involved in admissions, on-call management, and overnight care delivery
- ✓ The Person in Charge (PIC), Director, and Operational Governance Officer
- ✓ Any placement made at short notice (less than 24 hours) regardless of referral source

3 REFERRAL ACCEPTANCE CRITERIA

Chosanta Management will accept emergency placements where:

- ✓ The referral is received from a recognised statutory body (Tusla, An Garda Síochána, HSE) with appropriate authorisation
- ✓ Sufficient information is provided to conduct an immediate risk screening, including the child's age, gender, presenting concerns, and known risks
- ✓ The placement is compatible with the current resident group — assessed against risk of harm to the child and to existing residents
- ✓ Adequate staffing is available or can be arranged within a safe timeframe to meet the required supervision ratio
- ✓ The child's immediate medical, dietary, and safety needs can be reasonably accommodated

⚠ Important: A placement may be declined where acceptance would compromise the safety of existing residents or where staffing ratios cannot be maintained. All declined referrals must be documented with reasons and communicated to the referring body.

Emergency Placement Procedure

SECTION 2: EMERGENCY ADMISSIONS WORKFLOW

4 EMERGENCY ADMISSIONS WORKFLOW

Step-by-step process for all emergency placements:

- ✓ **Step 1 — Referral Receipt:** Referral received by PIC or on-call manager. Referral details logged immediately including: child's name, date of birth, referring social worker, nature of emergency, known risks, legal status, and contact information
- ✓ **Step 2 — Immediate Risk Screening:** PIC/on-call manager conducts a rapid placement compatibility assessment within 30 minutes of referral — considering current resident profiles, staffing levels, environmental capacity, and known risk factors
- ✓ **Step 3 — Placement Decision:** Decision to accept or decline is communicated to the referring body within 1 hour. If declined, reasons are documented and alternative support is offered where possible
- ✓ **Step 4 — Safeguarding Checks:** Prior to or immediately upon arrival — confirm legal authority for placement (court order, voluntary care agreement, Section 12 removal), verify identity of transporting personnel, and record the child's immediate presentation and demeanour
- ✓ **Step 5 — Arrival & Welcome:** Child is welcomed by a named keyworker; shown their room and communal areas; provided with immediate necessities (food, clothing, hygiene items); introduced to staff on shift in an age-appropriate manner
- ✓ **Step 6 — Immediate Needs Assessment:** Within 2 hours of arrival — assess and document medical needs, medication requirements, dietary requirements/allergies, emotional state, communication needs, and immediate safety concerns
- ✓ **Step 7 — Notifications:** PIC ensures the following notifications are completed within prescribed timeframes: Tusla Duty Social Worker (if not the referring body), parents/guardians where appropriate and safe, GP registration or out-of-hours medical if needed, and internal governance notification. A Significant Event Notification (SEN) must be filed in accordance with *Child Care (Standards in Children's Residential Centres) Regulations 1996*

5 OUT-OF-HOURS ESCALATION

Emergency placements received outside standard office hours (18:00–08:00 weekdays, weekends, and public holidays) follow the same workflow with the following additional requirements:

- ✓ The on-call manager is the primary decision-maker and must be contactable at all times via the designated on-call phone
- ✓ The Director must be notified within 1 hour of any out-of-hours emergency placement acceptance
- ✓ Additional staffing must be confirmed before the child arrives — no emergency placement proceeds with a single staff member on duty
- ✓ A full handover briefing must occur at the next shift change, with written notes provided to incoming staff
- ✓ The PIC must conduct a formal review of the placement by 12:00 on the next working day

Emergency Placement Procedure

SECTION 3: COMPATIBILITY, HEALTHCARE & STAFFING

6 PLACEMENT COMPATIBILITY ASSESSMENT

Before any emergency placement is confirmed, the PIC or on-call manager must complete a rapid compatibility assessment considering the following factors:

- ✓ **Age & developmental stage:** Suitability of the child's age relative to existing residents; avoidance of inappropriate age mixing
- ✓ **Gender considerations:** Bedroom allocation, bathroom arrangements, and supervision planning
- ✓ **Known risk factors:** History of aggression, self-harm, substance misuse, exploitation, or sexually harmful behaviour — assessed against current resident vulnerabilities
- ✓ **Legal status:** Consideration of any court orders, bail conditions, or restricted contact arrangements
- ✓ **Peer dynamics:** Known associations (positive or negative) between the incoming child and existing residents
- ✓ **Environmental capacity:** Physical space availability, bedroom suitability, and access to communal areas

7 HEALTHCARE ESCALATION

Immediate healthcare needs must be assessed and addressed as a priority during any emergency placement:

- ✓ Establish the child's GP details, current medications, allergies, and any chronic conditions from the referral information
- ✓ If the child arrives with prescribed medication — verify, record, and store securely in line with the Medication Management Policy
- ✓ If the child presents with injuries, signs of abuse, or acute illness — arrange immediate medical attention (GP, out-of-hours service, or A&E) and record observations
- ✓ If the child has mental health concerns or is in acute distress — contact CAMHS crisis team or local mental health services and implement continuous observation if required
- ✓ A healthcare assessment must be completed within 72 hours of placement, or sooner if clinically indicated

8 OVERNIGHT STAFFING ARRANGEMENTS

Emergency placements frequently occur during evening or overnight hours. The following staffing requirements apply:

- ✓ **Minimum staffing:** At least two staff members must be on duty in any centre accommodating a child overnight — one waking night staff, one additional support or sleeping-in staff
- ✓ **Staff competency:** At least one staff member on duty must have completed Children First training, fire safety, and first aid certification
- ✓ **Relief & agency staff:** May be deployed to meet ratios but must have completed Chosanta induction, Garda vetting, and be briefed on the specific child's immediate needs and risks
- ✓ **On-call escalation:** The on-call manager must remain available by phone throughout the first night of any emergency placement
- ✓ **Handover:** A detailed written and verbal handover must occur at shift change, covering the child's presentation, any incidents, and care instructions

Emergency Placement Procedure

SECTION 4: DOCUMENTATION, REVIEW & CHECKLIST

9 RECORD-KEEPING & DOCUMENTATION

- ✓ An Emergency Placement Record Form must be completed for every emergency admission — retained on the child's file and available for Tusla/HIQA inspection
- ✓ All decisions (accept/decline), risk assessments, and notifications must be documented contemporaneously with dates, times, and signatures
- ✓ A copy of the referral information and legal authority documentation must be filed within the child's placement record
- ✓ Any incident occurring during the first 72 hours must be reported through the standard Incident Reporting Procedure and flagged as placement-related

10 POST-PLACEMENT REVIEW

- ✓ **24-hour review:** PIC conducts an initial review of the placement — child's welfare, staff observations, immediate concerns, and contact with the referring social worker
- ✓ **72-hour review:** Formal placement review meeting involving PIC, keyworker, and (where possible) social worker — confirms suitability of ongoing placement, identifies care planning needs, and establishes an interim personal plan. Under **S.I. 259/1995 (Child Care Placement Regulations)** and the HIQA National Standards 2018, a statutory care plan must be prepared within **7 working days** of the child's admission. For emergency placements, an interim care plan is established at the 72-hour review, with the full statutory plan completed by day 7.
- ✓ **7-day review:** Full placement planning meeting — transition to a formal personal plan, risk assessment review, healthcare registration, education/activity arrangements, and family contact planning
- ✓ Reviews must be documented, filed, and shared with the referring body

✓ EMERGENCY PLACEMENT CHECKLIST

Item	Status	Completed By	Date/Time
Referral details received & logged			
Compatibility assessment completed			
Legal authority confirmed (court order / S.12 / VCA)			
Staffing arrangements confirmed			
Director/PIC notified			
Child arrived — welcome & immediate needs met			
Healthcare needs assessed			
Medication received, verified & stored			
Emergency Placement Record Form completed			
Tusla notification submitted			
24-hour review completed			
72-hour review completed			

PIC Signature: _____ **Date:** _____

On-Call Manager: _____ **Date:** _____

Legislative Basis: Child Care Act 1991 (s.12, s.13) · S.I. 259/1995 · Children First Act 2015 · HIQA National Standards 2018 (Standards 3, 5, 6) · Tusla SEA SOP

Policy Ref: CM-EPP-001 | **Review Date:** May 2027 | **Version:** 2.0